

METROPOLITAN UNIVERSITY HEART CENTER

Division of Cardiovascular Medicine & Inpatient Cardiology Services

DISCHARGE SUMMARY

Patient Name:	VANCE, ROBERT E.	MRN / FIN:	849-201-94 / 9940128
Date of Birth:	11/14/1957 (68 y/o M)	Admit / Disch Date:	10/12/2026 – 10/16/2026
Attending MD:	Marcus Vance, MD, FACC	Pref. Language:	English / Spanish

CLINICAL DIAGNOSES

PRIMARY: Acute Decompensated Heart Failure (ADHF) with reduced Ejection Fraction (HFrEF, LVEF 28%) [ICD-10: I50.21]
SECONDARY: Non-ST-Elevation Myocardial Infarction (NSTEMI) [I21.4]; Essential Stage 2 Hypertension [I10]; Dyslipidemia [E78.5].

HOSPITAL COURSE & CLINICAL NARRATIVE

The patient is a 68-year-old male with a history of hypertension who presented with NYHA Class III dyspnea on exertion, orthopnea (3-pillow), paroxysmal nocturnal dyspnea (PND), and worsening bilateral lower extremity pitting edema (+3). Cardiac biomarkers showed peak Troponin I elevation to 4.25 ng/mL and initial N-terminal pro-BNP of 1,850 pg/mL. Bedside Transthoracic Echocardiogram (TTE) demonstrated left ventricular systolic dysfunction with an estimated LVEF of 28% and diffuse anterior hypokinesis. Emergency cardiac catheterization revealed 75% proximal LAD stenosis; patient successfully underwent Percutaneous Coronary Intervention (PCI) with a single drug-eluting stent (DES). Patient was aggressively diuresed with IV diuretics (Lasix) achieving a net negative fluid balance of 4.2 Liters with resolution of pulmonary rales and dyspnea. Renal function and serum electrolytes remained stable throughout hospitalization.

DISCHARGE MEDICATIONS & PRESCRIPTION REGIMEN

Medication & Generic	Dose & Route	Frequency	Clinical Indication	Special Patient Instructions
ENTRESTO <i>Sacubitril / Valsartan</i>	24 / 26 mg PO	BID (Twice Daily)	HFrEF (ARNI Therapy)	Do not take within 36 hours of ACE inhibitors. Monitor BP daily.
JARDIANCE <i>Empagliflozin</i>	10 mg PO	QD (Every Morning)	Cardioprotection / SGLT2i	Ensure adequate oral hydration. Maintain strict foot hygiene.
COREG <i>Carvedilol</i>	6.25 mg PO	BID (Twice Daily)	Beta-Blocker / Heart Rate	Take with food. Do not stop abruptly. Hold if heart rate < 55 bpm.
LASIX <i>Furosemide</i>	40 mg PO	QD (Every Morning)	Loop Diuretic / Fluid Balance	Take in the morning to prevent nocturnal diuresis. Weigh daily.
PLAVIX <i>Clopidogrel</i>	75 mg PO	QD (Once Daily)	Antiplatelet (Post-Stent)	Dual Antiplatelet Therapy (DAPT) for 12 months. Do NOT skip doses.
ECOTRIN <i>Aspirin EC</i>	81 mg PO	QD (Once Daily)	Antiplatelet / CAD	Take with food. Watch for signs of gastrointestinal bleeding.

- ⚠ WARNING SIGNS — WHEN TO SEEK IMMEDIATE MEDICAL ATTENTION**
- **Sudden Weight Gain:** Weight increase of > 2 lbs in 24 hours or > 4 lbs in 7 consecutive days.
 - **Worsening Respiratory Symptoms:** Inability to lie flat without extra pillows, severe shortness of breath at rest, or nocturnal gasping.
 - **Systemic Signs:** Increasing swelling in feet, ankles, or abdomen, chest tightness, dizziness, or lightheadedness.

OUTPATIENT FOLLOW-UP & LAB ORDERS

Provider / Specialty	Appointment Date & Time	Location / Clinic	Required Pre-Visit Testing
Dr. Marcus Vance (Cardiology)	10/24/2026 at 10:30 AM	Heart Center, Suite 400	Basic Metabolic Panel (BMP) & K+ level in 5 days
Primary Care Physician (PCP)	11/02/2026 at 02:00 PM	Internal Medicine Associates	Routine Blood Pressure Log review

Electronically Signed by: Marcus Vance, MD, FACC
Attending Physician • Department of Cardiology • NPI: 1948201948

Date/Time: 10/16/2026 14:15 EST

Patient Name:	ROSTOVA, ELENA	MRN / FIN:	512-903-11 / 8810932
Date of Birth:	04/22/1954 (72 y/o F)	Admit / Disch Date:	10/14/2026 – 10/18/2026
Attending MD:	Sarah Jenkins, MD, FCCP	Pref. Language:	Russian / English

CLINICAL DIAGNOSES

PRIMARY: Acute Exacerbation of Chronic Obstructive Pulmonary Disease (AECOPD) [ICD-10: J44.1]
SECONDARY: Right Lower Lobe Community-Acquired Pneumonia (CAP) [J18.9]; Acute Hypoxemic Respiratory Failure (resolved) [J96.01]; Tobacco Use Disorder (former).

HOSPITAL COURSE & CLINICAL NARRATIVE

The patient is a 72-year-old female with severe baseline COPD who presented with severe dyspnea, acute bronchospasm, and production of thick mucopurulent sputum. On arrival, arterial blood gas (ABG) revealed pH 7.31, PaCO2 54 mmHg, PaO2 58 mmHg, consistent with acute respiratory acidosis with hypoxemia. Initial oxygen saturation (SpO2) was 85% on room air. Chest X-ray (CXR) demonstrated right lower lobe (RLL) alveolar consolidation and focal atelectasis. Peripheral CBC showed marked leukocytosis (WBC 16.8 x 10^3/μL) with a left shift. Patient was treated with IV methylprednisolone, broad-spectrum IV Ceftriaxone, and continuous nebulized SABA/SAMA bronchodilators. Clinical symptoms improved significantly over 48 hours. Sputum cultures showed normal oral flora. Oxygen weaning was completed successfully, and patient is discharged on room air with baseline SpO2 93%.

DISCHARGE MEDICATIONS & PRESCRIPTION REGIMEN

Medication & Generic	Dose & Route	Frequency	Clinical Indication	Special Patient Instructions
LEVAQUIN Levofloxacin	750 mg PO	QD x 5 days	Fluoroquinolone Antibiotic / CAP	Complete entire 5-day oral course. Drink plenty of fluids. Report tendon pain.
PREDNISONE Prednisone Taper	40mg / 20mg / 10mg PO	QD (3 days per dose)	Systemic Corticosteroid / COPD	Take with breakfast. Taper schedule: 40mg x3d, 20mg x3d, 10mg x3d, then STOP.
ADVAIR DISKUS Fluticasone / Salmeterol	250 / 50 mcg	1 puff Inhaled BID	ICS / LABA Maintenance	Rinse mouth thoroughly with water and spit after each dose to prevent thrush.
COMBIVENT RESPIMAT Albuterol / Ipratropium	20 / 100 mcg	1-2 puffs Q4H PRN	Rescue SABA / SAMA	Use for acute shortness of breath or wheezing. Do not exceed 8 puffs/day.
MUCINEX ER Guaifenesin ER	600 mg PO	BID PRN	Mucolytic / Expectorant	Take with a full glass of water to help thin and loosen bronchial secretions.

- ⚠ WARNING SIGNS — WHEN TO SEEK IMMEDIATE MEDICAL ATTENTION**
- Respiratory Distress:** Increased work of breathing, intercostal retractions, or inability to speak full sentences.
 - Sputum Changes:** Marked increase in sputum volume or change in color to dark green or rust-brown.
 - Fever & Hypoxia:** Recurrent oral temperature > 101.0°F or home pulse oximeter SpO2 dropping below 88%.

OUTPATIENT FOLLOW-UP & LAB ORDERS

Provider / Specialty	Appointment Date & Time	Location / Clinic	Required Pre-Visit Testing
Pulmonary Clinic (Dr. S. Jenkins)	11/01/2026 at 09:15 AM	St. Jude Respiratory Clinic, Suite 210	Repeat Chest X-Ray 2 hours prior to visit
Pulmonary Rehabilitation	11/08/2026 at 11:00 AM	Outpatient Rehab Center	Spirometry / PFT Evaluation

Electronically Signed by: Sarah Jenkins, MD, FCCP
Attending Physician • Department of Pulmonary Medicine • NPI: 1827401928
Date/Time: 10/18/2026 11:30 EST

Patient Name:	MENDEZ, CARLOS R.	MRN / FIN:	304-771-82 / 7710294
Date of Birth:	09/05/1981 (45 y/o M)	Admit / Disch Date:	10/15/2026 – 10/19/2026
Attending MD:	Aris Thorne, MD	Pref. Language:	Spanish / English

CLINICAL DIAGNOSES

PRIMARY: Acute Gallstone Pancreatitis (Resolved) [ICD-10: K85.10]
SECONDARY: Type 2 Diabetes Mellitus with Uncontrolled Hyperglycemia (HbA1c 11.4%) [E11.69]; Cholelithiasis [K80.20]; Severe Hypertriglyceridemia [E78.1].

HOSPITAL COURSE & CLINICAL NARRATIVE

The patient is a 45-year-old male presenting with severe mid-epigastric pain radiating to the back, associated with intractable nausea and emesis. Diagnostic laboratory findings confirmed acute pancreatitis with **hyperamylasemia (Amylase 890 U/L)** and dramatic **hyperlipasemia (Lipase 2,450 U/L)**. Abdominal ultrasound demonstrated multiple gallstones (**cholelithiasis**) with acoustic shadowing, without evidence of **choledocholithiasis** or biliary ductal dilation. Patient was initially made strictly **NPO (nil per os)** and resuscitated with aggressive intravenous isotonic crystalloids. Initial capillary blood glucose was severely elevated at 342 mg/dL with an admission **HbA1c of 11.4%**. Endocrine consultation was obtained; patient was initiated on a subcutaneous **basal-bolus insulin regimen**. Abdominal pain resolved completely, pancreatic enzymes normalized, and diet was successfully advanced to a low-fat solid diet. Patient is cleared for discharge with outpatient surgical follow-up.

DISCHARGE MEDICATIONS & PRESCRIPTION REGIMEN

Medication & Generic	Dose & Route	Frequency	Clinical Indication	Special Patient Instructions
LANTUS <i>Insulin Glargine</i>	24 Units SC	QHS (At Bedtime)	Basal Insulin / Type 2 DM	Inject subcutaneously once daily at bedtime. Rotate injection sites daily.
HUMALOG <i>Insulin Lispro</i>	8 Units SC	TID (Before Meals)	Prandial Rapid-Acting Insulin	Inject within 15 mins before meals. Use sliding scale for BG > 180 mg/dL.
GLUCOPHAGE <i>Metformin</i>	500 mg PO	BID (With Meals)	Biguanide / Antidiabetic	Take with breakfast and dinner to minimize gastrointestinal upset.
TRICOR <i>Fenofibrate</i>	145 mg PO	QD (Once Daily)	Fibrate / Hypertriglyceridemia	Take daily with food. Helps prevent hypertriglyceridemia-induced pancreatitis.
ZOFRAN <i>Ondansetron</i>	4 mg PO	Q8H PRN Nausea	Antiemetic / Nausea Control	Dissolve on tongue or swallow with water as needed for persistent nausea.

- ⚠ WARNING SIGNS — WHEN TO SEEK IMMEDIATE MEDICAL ATTENTION**
- Recurrent Pancreatitis Symptoms:** Severe upper abdominal pain radiating straight to the back, accompanied by nausea or vomiting.
 - Hypoglycemia (Low Blood Sugar):** Shaking, sweating, confusion, fast heart rate (BG < 70 mg/dL). Consume 15g fast-acting sugar immediately.
 - Severe Hyperglycemia:** Blood glucose persistently > 300 mg/dL, excessive thirst, frequent urination, or fruity breath odor.

OUTPATIENT FOLLOW-UP & LAB ORDERS

Provider / Specialty	Appointment Date & Time	Location / Clinic	Required Pre-Visit Testing
General Surgery (Dr. H. Rodriguez)	11/04/2026 at 01:30 PM	Surgical Specialists Suite 300	Consultation for elective Laparoscopic Cholecystectomy
Endocrinology & Diabetes Clinic	11/12/2026 at 10:00 AM	Diabetes Center, 2nd Floor	Fasting Blood Glucose Log & Lipid Panel

Electronically Signed by: Aris Thorne, MD
Attending Physician • Department of Gastroenterology • NPI: 1402938102

Date/Time: 10/19/2026 16:45 EST

Patient Name:	TANAKA, HIROSHI	MRN / FIN:	918-334-05 / 4491023
Date of Birth:	02/18/1952 (74 y/o M)	Admit / Disch Date:	10/17/2026 – 10/21/2026
Attending MD:	Maya Lin, MD	Pref. Language:	Japanese / English

CLINICAL DIAGNOSES

PRIMARY: Acute Ischemic Cerebrovascular Accident (CVA) in Left MCA Territory [ICD-10: I63.512]
SECONDARY: Paroxysmal Non-Valvular Atrial Fibrillation (CHA2DS2-VASc Score = 5) [I48.0]; Mild Expressive Aphasia & Right Hemiparesis (improving) [R47.01 / G81.91]; Hyperlipidemia [E78.5].

HOSPITAL COURSE & CLINICAL NARRATIVE

The patient is a 74-year-old male who presented with acute onset of right-sided weakness and expressive aphasia. Initial National Institutes of Health Stroke Scale (NIHSS) score on presentation was 9. Non-contrast cranial CT scan excluded acute hemorrhage. The patient met criteria and received intravenous thrombolytic therapy (tPA / Alteplase) within the 4.5-hour therapeutic window, resulting in significant neurological recovery (NIHSS improved to 2). Brain MRI confirmed a subcortical infarct in the left Middle Cerebral Artery (MCA) distribution. Continuous cardiac telemetry documented intermittent paroxysmal atrial fibrillation, establishing an cardioembolic stroke etiology (CHA2DS2-VASc score = 5). Carotid Doppler demonstrated <30% non-stenotic atherosclerosis. Bedside swallow evaluation was passed. Patient has been started on full oral anticoagulation with an oral direct thrombin/Xa inhibitor and is cleared for discharge with home physical and speech therapy.

DISCHARGE MEDICATIONS & PRESCRIPTION REGIMEN

Medication & Generic	Dose & Route	Frequency	Clinical Indication	Special Patient Instructions
ELIQUIS <i>Apixaban</i>	5 mg PO	BID (Twice Daily)	DOAC / Stroke Prevention AFib	Take exactly every 12 hours. DO NOT SKIP DOSES. Watch for excessive bleeding.
LIPITOR <i>Atorvastatin</i>	80 mg PO	QHS (At Bedtime)	High-Intensity Statin Therapy	Take at bedtime. Report unexplained muscle soreness or dark urine immediately.
ZESTRIL <i>Lisinopril</i>	10 mg PO	QD (Every Morning)	ACE Inhibitor / BP Control	Maintain blood pressure target < 130/80 mmHg. Report dry cough or facial swelling.
KEPPRA <i>Levetiracetam</i>	500 mg PO	BID x 30 days	Anticonvulsant Prophylaxis	Prophylactic therapy post-cortical infarct. Do not stop abruptly without MD guidance.
PRILOSEC <i>Omeprazole</i>	20 mg PO	QD (Before Breakfast)	PPI / Gastric Protection	Take 30 minutes before the first meal of the day with water.

- ⚠ WARNING SIGNS — FAST STROKE ALERTS & BLEEDING RISKS**
- FAST Symptoms:** Face drooping, Arm weakness, Speech difficulty → Call 911 IMMEDIATELY.
 - Bleeding Warnings (On Anticoagulation):** Black tarry stools, blood in urine, unexplained bruising, severe unremitting headache, or bleeding from gums.
 - Fall Prevention:** High risk of fall injury due to leg weakness. Use mobility assistance device (walker).

OUTPATIENT FOLLOW-UP & REHABILITATION ORDERS

Provider / Specialty	Appointment Date & Time	Location / Clinic	Required Pre-Visit Testing
Stroke Neurology Clinic (Dr. M. Lin)	11/03/2026 at 11:00 AM	Neuroscience Institute, 5th Floor	Neurological Exam & BP Log
Home Health PT / OT / Speech Therapy	Starts 10/23/2026 (In-Home)	Home Visit Services	Comprehensive Mobility & Language Evaluation

Electronically Signed by: Maya Lin, MD
Attending Physician • Department of Vascular Neurology • NPI: 1039482019

Date/Time: 10/21/2026 13:20 EST

VALLEY HEALTH NEPHROLOGY & INFECTIOUS CARE

Department of Nephrology, Urology & Antimicrobial Stewardship

DISCHARGE SUMMARY

Patient Name:	AL-MANSOOR, FATIMA	MRN / FIN:	621-009-43 / 3302914
Date of Birth:	08/12/1967 (59 y/o F)	Admit / Disch Date:	10/18/2026 – 10/22/2026
Attending MD:	David K. O'Connor, MD	Pref. Language:	Arabic / English

CLINICAL DIAGNOSES

PRIMARY: Acute Pyelonephritis (Left Kidney) [ICD-10: N10]
SECONDARY: Acute Kidney Injury (AKI Stage II) secondary to Prerenal Azotemia & Sepsis (Resolved) [N17.9]; Escherichia coli Bacteremia [A41.51]; Dysuria [R30.0].

HOSPITAL COURSE & CLINICAL NARRATIVE

The patient is a 59-year-old female admitted with high-grade fever (103.1°F), chills, severe left **costovertebral angle (CVA) tenderness**, dysuria, and flank pain. Laboratory evaluation was notable for severe **leukocytosis (WBC 19.2 x 10³/μL)** and **Acute Kidney Injury (AKI Stage II)** with serum creatinine elevated to 2.35 mg/dL (baseline 0.9 mg/dL) and elevated **BUN of 42 mg/dL**. Urinalysis demonstrated gross **pyuria (>100 WBC/hpf)** and positive nitrites. Blood and urine cultures confirmed **Escherichia coli bacteremia** sensitive to fluoroquinolones. Patient met criteria for sepsis and was fluid resuscitated with IV isotonic fluids and treated with IV Ceftriaxone 1g Q24H. Renal function steadily recovered with serum creatinine decreasing to 1.05 mg/dL. Patient has been afebrile for >48 hours with complete resolution of flank pain and is stepped down to an oral antimicrobial regimen.

DISCHARGE MEDICATIONS & PRESCRIPTION REGIMEN

Medication & Generic	Dose & Route	Frequency	Clinical Indication	Special Patient Instructions
CIPRO <i>Ciprofloxacin</i>	500 mg PO	BID x 7 days	Oral Antibiotic / E. coli Pyelonephritis	Complete 7 full days to total 14d antibiotic course. Do not take with antacids or milk.
PYRIDIUM <i>Phenazopyridine</i>	200 mg PO	TID PRN (Max 2 days)	Urinary Analgesic / Dysuria	Take after meals. NOTE: Will cause harmless ORANGE/RED discoloration of urine/tears.
TYLENOL <i>Acetaminophen</i>	650 mg PO	Q6H PRN Fever/Pain	Antipyretic / Analgesic	For temperature > 100.4°F or mild pain. Do not exceed 3,000 mg in 24 hours.
MIRALAX <i>Polyethylene Glycol 3350</i>	17 g PO	QD PRN Constipation	Osmotic Laxative	Dissolve 1 capful in 8 oz of water or juice daily as needed.

- ⚠ WARNING SIGNS & ABSOLUTE MEDICATION CONTRAINDICATIONS**
- ABSOLUTE CONTRAINDICATION:** Avoid ALL NSAID pain relievers (Ibuprofen, Advil, Motrin, Aleve, Naproxen, Meloxicam) due to high risk of acute kidney injury recurrence!
 - Infection Recurrence:** Return of fever (> 100.4°F), shaking chills, worsening flank/back pain, or inability to tolerate oral fluids.
 - Hydration Requirement:** Drink 2.5 to 3.0 Liters of water daily to maintain renal perfusion and urinary flushing.

OUTPATIENT FOLLOW-UP & LAB ORDERS

Provider / Specialty	Appointment Date & Time	Location / Clinic	Required Pre-Visit Testing
Primary Care Physician (PCP)	10/29/2026 at 09:00 AM	Valley Primary Care Clinic	Repeat BMP (Creatinine/BUN) & Urine Culture 1 day prior
Nephrology Follow-up	11/15/2026 at 02:15 PM	Valley Nephrology Suite 105	Serum Creatinine & Urinalysis check

Electronically Signed by: David K. O'Connor, MD
Attending Physician • Department of Nephrology • NPI: 1192847103

Date/Time: 10/22/2026 10:15 EST